PatientKeeper

RETORNO

From a missed follow-up
to a clear next step.



Athos Felipe · FIGUEIRA AI

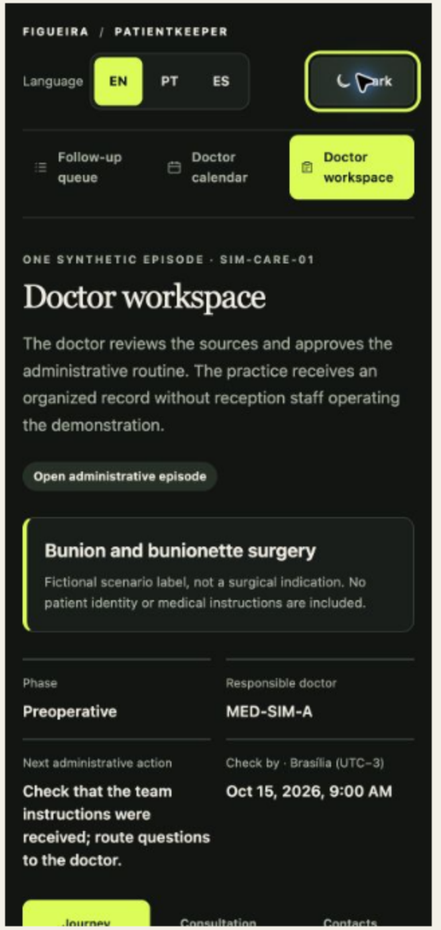
A missed follow-up leaves a next step open

An administrative problem drawn from everyday work with the doctors we serve.

An unfinished follow-up still needs a next step

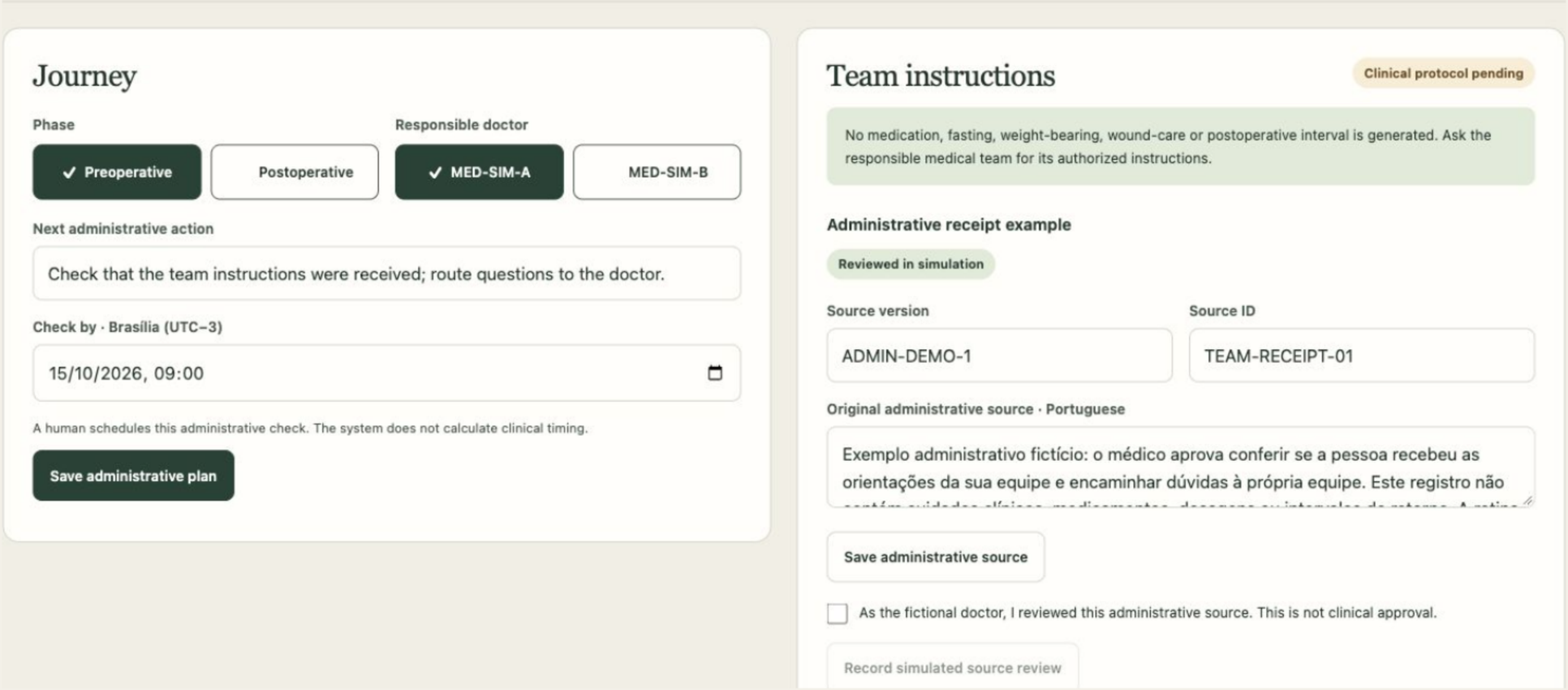
The doctor needs to see what remains open and which source supports the next check.

Built from Athos’s everyday work with the doctors FIGUEIRA serves.



The doctor plans the next step

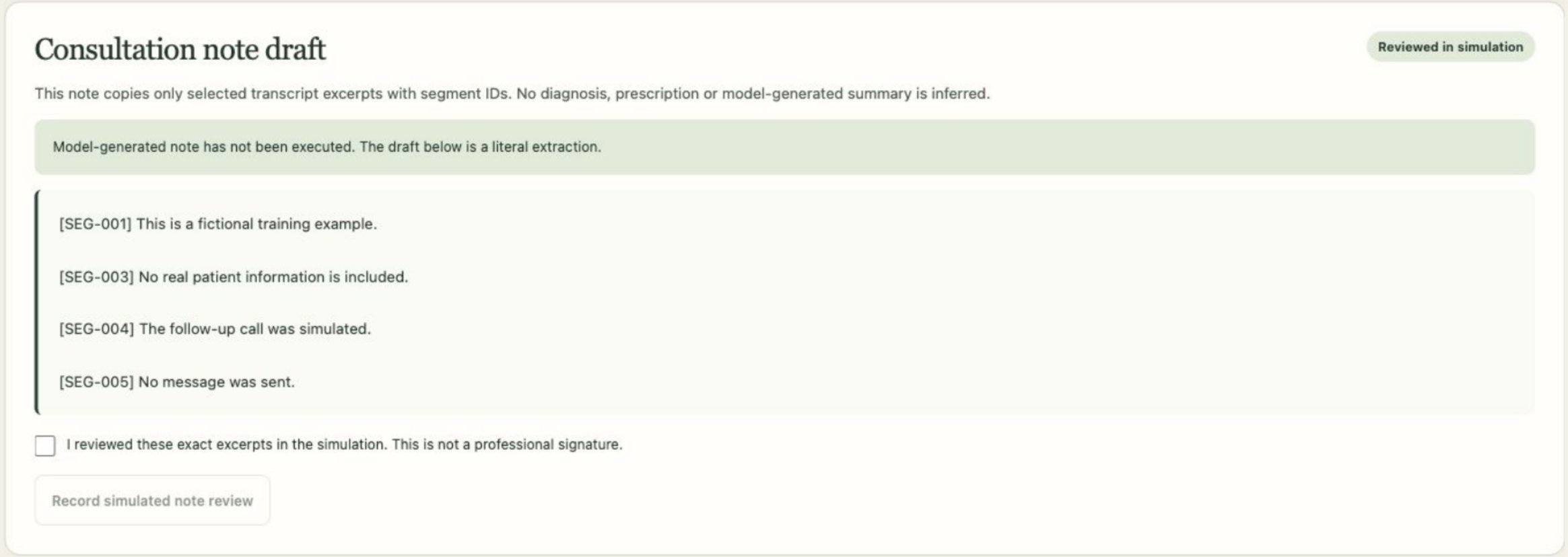
Preoperative and postoperative labels organise one administrative episode.



The doctor sets the administrative check. Clinical care instructions remain pending.

Audio becomes a source the doctor can inspect

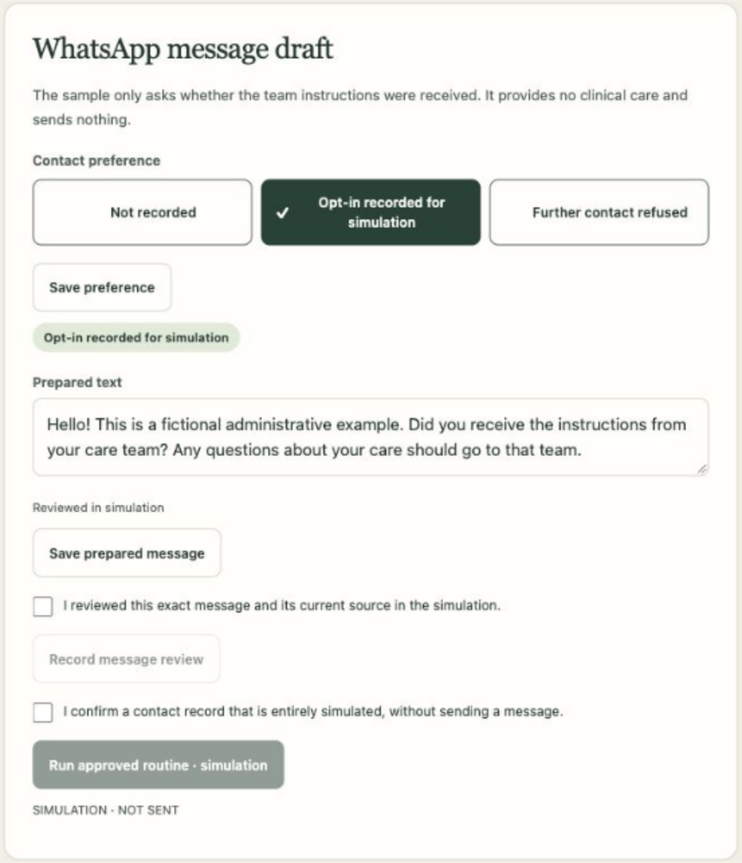
Local CPU transcription supports a draft made from selected literal segments.



Local CPU version. Public demo: audio disabled. The draft uses selected literal segments.

Reviewed contact remains a simulation

Recorded consent and the current source gate a message. A fictional reply supports human resolution.





A source change invalidates review. Refusal blocks contact. No external message is sent.

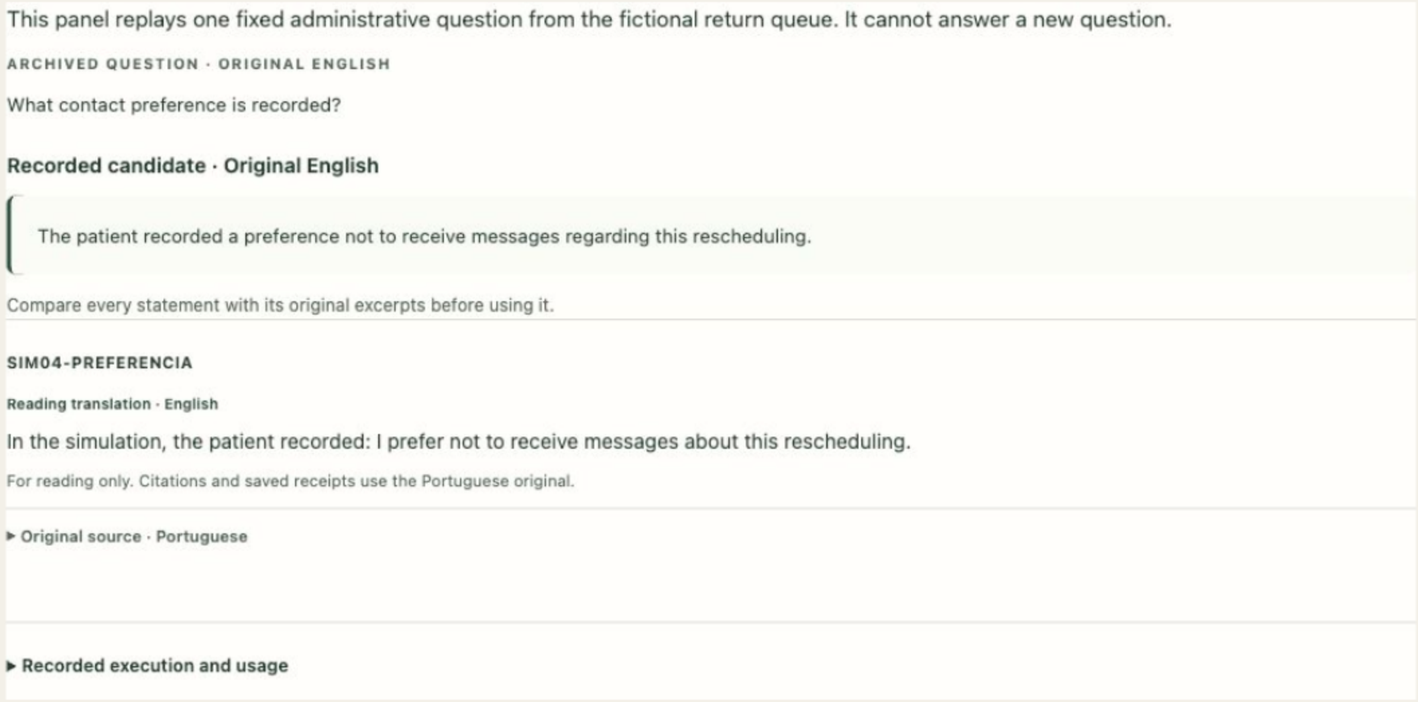
Gemma on AMD Token Factory

Five distinct queries on the original return queue. The public panel replays recorded results.

5 queries: 3 answer candidates · 1 abstention · 1 timeout

SIM04-PREFERENCIA
Original source · Portuguese

“Na simulacao, o paciente registrou: prefiro nao receber mensagens sobre esta remarcacao.”



Original queue evidence. No care-note generation or live demo inference.

Sources and consent keep the limits visible

Human checks control the next step. Clinical protocol content remains pending.

Refusal blocks contact

A changed source invalidates review

Conflicts and clinical questions go to the responsible team

The export remains a draft for human review

Authorised surgical instructions remain pending.

5 October

Declared pre-event administrative scaffold.

12 October

Source review and real Qwen execution on AMD.
Recorded playback and interface improvements.

14 October

Doctor workspace and local CPU ASR.
Gemma service tests and recorded evidence.

Time recovered for patient care

ILLUSTRATIVE SAVING, NOT MEASURED

30 min/day

6 records/day × (15 min manual
− 10 min assisted)

15 min: team-reported manual baseline.
10 min: assumed assisted total, including preparation, doctor review and corrections.

INFORMAL REPORT: ANOTHER TOOL

A physician reported saving about 2 hours a day with another transcription tool.

Informal report shared with our team, not a PatientKeeper result.

PatientKeeper savings remain unmeasured. Local ASR supports short dictation up to 120 seconds.
Notes require doctor review.

A next step the practice can review

Hypothesis: clearer handoffs and fewer unfinished administrative checks.

The doctor retains control.
The practice receives a source-linked draft
and visible pending work.

Technical evidence: real local ASR, persisted fictional state and recorded AMD queries.
Operational savings and clinical outcomes remain unmeasured.

PUBLIC DEMO

athosfif.github.io/patientkeeper-retorno-act-iii/

SOURCE CODE

github.com/athosfif/patientkeeper-retorno-act-iii

Next evaluation: completion time and rework in comparable fictional tasks.